

# Health-Related Emoji Coverage as a Standards-Maintenance Issue

Title: Health-Related Emoji Coverage as a Standards-Maintenance Issue

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Date: 2026-07-10

Status: Individual Contribution; affiliations are provided for identification

Action: For consideration by UTC; refer to ESR

**Status:** Discussion document for referral to the Unicode Technical Committee (UTC) and the Emoji Standard and Research Working Group (ESR).

## 0.1 Executive summary

Health-related emoji coverage presents a coherent review question. Health concepts are distributed across anatomy, diagnostic, laboratory, medication, emergency-care, and clinical-equipment contexts. This document uses “standards maintenance” narrowly to mean review of existing emoji organization, coverage analysis, and proposal-handling guidance. It does not mean automatic addition of characters.

This document asks the UTC to authorize ESR to prepare a report describing how existing health-related emoji are represented in Unicode emoji data and ordering, whether the current organization creates a material discoverability problem, how the existing Emoji selection factors apply to health-related concepts, and what procedural guidance, if any, should be reported to the UTC.

This document addresses review and organization. Individual concepts remain subject to the current Unicode Emoji proposal requirements.

## 0.2 Why now

The 2026 Emoji proposal window is open through July 31, 2026. Several health-related concepts have prior proposal history, and current submissions are being prepared under the evidence, image-rights, and submission requirements in force for this cycle. The current Unicode Emoji Ordering data provides a reproducible starting point for reviewing where health-related concepts appear today.

## 0.3 Concrete UTC action requested

We request that the UTC authorize the Emoji Standard and Research Working Group to prepare a report on health-related emoji coverage and return recommendations on organization, application of the existing selection factors, and procedural handling of individual proposals. The review mandate should address the following five questions:

1. **Recognize health-related emoji coverage as a coherent review question.** The domain should be assessed as a connected coverage problem even when its existing characters remain in different keyboard groups.
2. **Review how the domain is organized.** The review should examine anatomy, diagnostics, laboratory medicine, medication, emergency care, and clinical equipment, including the relationship between existing emoji and missing concepts.

3. **Assess how the existing Emoji selection factors apply.** The review should consider usage evidence, distinctiveness, multiple durable meanings, combinability, category completion, non-duplication, visual legibility, and the exclusion factors already published by Unicode.
4. **Refer individual concepts to the normal Emoji proposal process.** A coverage review must not become a bundled encoding request. Individual concepts must be evaluated in their own complete submissions.
5. **Clarify optional vendor input.** Vendors may provide non-binding comments on visual legibility, accessibility, platform-neutrality, and implementation considerations. Submitters remain responsible for evidence, licensing, and proposal completeness.

The requested outcome is an ESR report and recommendation on the review framework. It is not a request for automatic approval of any character.

## 0.4 Review structure

**Standards review.** ESR reviews the organization of existing health-related emoji, the six-domain coverage framework, the application of existing selection factors, and any process recommendations that should be reported to the UTC.

**Individual proposals.** Each candidate is prepared and submitted as a separate, complete Unicode Emoji proposal. The standards review does not pre-approve any candidate. Illustrative examples are listed in Appendix A; they are not requested for decision by this document.

## 0.5 Scope of the maintenance review

The review should use a coverage matrix with six domains:

**Anatomy.** Brain, anatomical heart, lungs, tooth, and mechanical limbs; assess whether major internal organs and systems are represented distinctly enough for ordinary communication.

**Diagnostics.** X-ray, stethoscope, and thermometer-related face; assess common modalities and readings without over-specific device branding.

**Laboratory medicine.** Test tube, petri dish, DNA, and microbe; assess whether specimen, cell, blood, and laboratory concepts are organized coherently.

**Medication.** Pill, syringe, and drop of blood; assess whether Pill Box and Inhaler are independently useful and visually distinct.

**Emergency care.** Adhesive bandage, crutch, ambulance, and hospital; assess first-response objects and care actions as broad paradigms rather than logos.

**Clinical equipment.** Stethoscope, x-ray, and crutch; identify equipment concepts with broad public use, clear silhouettes, and no existing substitute.

The matrix organizes the review; it does not imply that every gap should produce a character.

## 0.6 Prioritization framework

Each candidate should receive a documented assessment against the following factors, with evidence attached to its own proposal:

1. **Independent concept:** the candidate represents a recognizable entity or activity not already represented by an existing emoji or sequence.
2. **Multiple durable uses:** the concept supports ordinary, educational, public-health, clinical, or metaphorical communication beyond one narrow diagnosis or campaign.
3. **Distinctive visual paradigm:** a non-specialist should identify it at 18x18 without relying on text, a logo, or exact vendor artwork.

4. **Evidence of usage:** the proposal supplies the current Unicode evidence set and explains query disambiguation and reproducibility.
5. **Category completion:** the candidate fills a meaningful gap in an existing family without arguing for an open-ended taxonomy.
6. **Combinaability:** the candidate supports useful sequences with existing emoji, while no sequence is requested as part of encoding.
7. **Non-duplication and exclusion control:** the proposal addresses already represented, overly specific, open-ended, transient, and existing-emoji-substitute concerns.
8. **Accessibility:** color is not the only carrier of meaning; the silhouette and essential cues survive small-size and black-and-white review.
9. **Rights readiness:** all submitted example images have an ownership, assignment, public-domain, or compatible open-license chain.
10. **Implementation readiness:** vendors can evaluate the paradigm without being asked to adopt a prescribed design.

No single factor decides a candidate. A high-profile medical endorsement cannot substitute for public frequency evidence, and a high search count cannot cure an indistinct or overly specific design.

## 0.7 Relationship to individual submissions

Each individual submission must be complete on its own and publicly accessible as a PDF. This L2 explains the domain-level review; it does not replace evidence for an individual candidate.

For each proposed concept, the packet must include the required title, submitter and date, identification, keywords and category, images, inclusion factors, exclusion factors, other information, and the image-rights warranty required by the current Unicode guidelines. The packet must include its own frequency screenshots for Google Search, Google Video Search, Google Trends Web Search, Google Trends Image Search, and Google Books Ngram Viewer, or a clearly justified reproducible substitute where a required source is unavailable.

Every submission must be complete: no TODOs, placeholders, pending screenshots, unresolved rights statements, or unverified eligibility dates.

## 0.8 Vendor participation model

Vendor participation should be optional, transparent, non-binding, and non-exclusive. It may include:

- review whether a candidate remains legible at common emoji sizes;
- test color and black-and-white paradigms for accessibility;
- identify ambiguous cues, misleading clinical details, and platform-neutral alternatives;
- provide anticipated implementation considerations; and
- return technical comments through the normal UTC/ESR process.

Submitters remain responsible for Unicode evidence, licensing, and proposal completeness. Vendor review does not request exact artwork or commit a vendor to ship an encoded character.

## 0.9 Requested referral and working-group action

We request that this document be routed to ESR for the report described above. Any associated individual proposal materials should be reviewed through the ordinary Emoji process under the published criteria. The affiliations listed in this document identify the authors; they do not replace the evidence required for an individual proposal.

The requested working-group output is a report and recommendation on organization, application of the existing selection factors, and procedural handling of individual concepts.

## **o.10 Boundaries and safeguards**

- The review does not require creation of a top-level Health group.
- The review does not propose a comprehensive anatomy taxonomy.
- Individual concepts will use the normal Emoji proposal form.
- Professional society letters provide domain context; they are not frequency evidence.
- Medical prevalence and clinical importance do not replace Unicode's required usage evidence.
- Previously declined concepts require an eligibility check immediately before filing.
- Submitted artwork is a reference paradigm, not required vendor artwork.

## **o.11 Appendix A: Illustrative concepts for the separate proposal track**

The following concepts illustrate the types of gaps that may be examined in separate submissions. This list is not a bundled request, a priority ranking, or a request for action by the UTC in this document:

- Anatomy: kidney, liver, stomach.
- Diagnostics and laboratory medicine: EKG/ECG and white blood cell.
- Medication: Pill Box and Inhaler.
- Emergency care and clinical equipment: First Aid Kit, Blood Bag, IV Bag, Leg Cast, and Weight Scale.

## **o.12 Appendix B: Factual basis for the coverage review**

The coverage review should begin with the current Unicode Emoji Ordering data and the current emoji list. The authors' working inventory records each relevant emoji's Unicode name, code point, group, subgroup, and rationale for inclusion in the health-related review. The inventory is a starting point for ESR review, not a claim that Unicode should move or encode every listed item.

Current source data:

- Unicode Emoji Ordering: <https://unicode.org/emoji/charts/emoji-ordering.html>
- Unicode Emoji List: <https://unicode.org/emoji/charts/emoji-list.html>
- Unicode Standard Annex #51, Unicode Emoji: <https://www.unicode.org/reports/tr51/>

The review should verify the inventory against the current released data before relying on counts or group assignments.

## **o.13 References**

- Unicode, Guidelines for Submitting Unicode Emoji Proposals: <https://www.unicode.org/emoji/proposals.html>
- Unicode, Unicode Standard Annex #51, Unicode Emoji: <https://www.unicode.org/reports/tr51/>
- Unicode, Emoji Ordering: <https://unicode.org/emoji/charts/emoji-ordering.html>
- Unicode, Emoji Proposals Status: <https://unicode.org/emoji/emoji-proposals-status.html>
- Unicode, Emoji Standard and Research Working Group Report for UTC #186: <https://www.unicode.org/L2/L2026/26008r-esr-report-utc186.pdf>
- Unicode, Emoji Submission FAQ: [https://www.unicode.org/faq/emoji\\_submission.html](https://www.unicode.org/faq/emoji_submission.html)
- Unicode, Emoji Proposal Agreement and License: <https://www.unicode.org/emoji/emoji-proposal-agreement.pdf>
- Unicode, Pending Document Submission: <https://www.unicode.org/pending/docsubmit.html>
- Unicode, Emoji Submission Form: <https://forms.gle/6KSiyHrUdBkTMNaB8>